Panitumumab + FOLFIRI

A 14-day cycle: panitumumab, irinotecan, folinic acid and fluorouracil start on Day 1, followed by a fluorouracil pump for about 46 hours. Arrange disconnection around Day 3. Continue according to benefit and tolerance.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Eyes

Panitumumab

New eye pain or visual change needs prompt advice.

2 Mouth

Fluorouracil + folinic acid

Report ulcers or pain affecting eating and drinking.

3 Chest

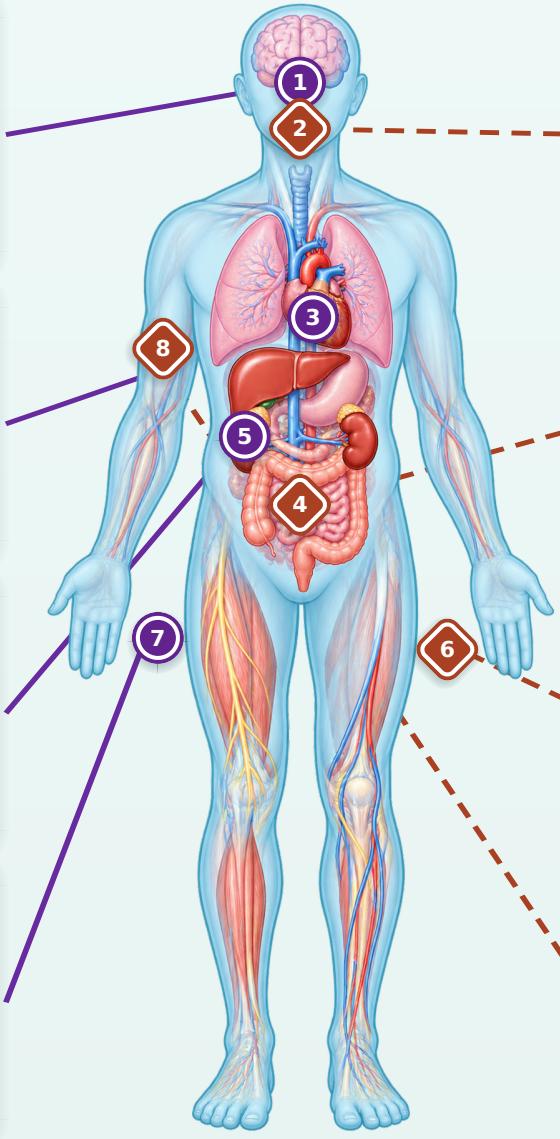
Fluorouracil

New chest pain or breathlessness needs urgent assessment.

4 Bowel

Irinotecan / fluorouracil

Report diarrhoea, increased stoma output or severe abdominal pain.



5 Hydration / minerals

Panitumumab + chemotherapy

Reduced urine, cramps or weakness need advice; keep your blood-test appointments.

6 Skin / nails

Panitumumab

Report rash, cracks or painful nails early.

7 Hands / feet

Fluorouracil

Report painful redness, peeling or difficulty using your hands or walking.

8 Pump / line

Treatment delivery

Pain, leakage, swelling or a loose connection needs immediate advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

Your treatment, at home.

Important medicine-specific effects

Panitumumab

- Rash, dry skin, cracks and sore nails
- Eye inflammation or corneal injury
- Low magnesium, calcium or potassium
- Diarrhoea, dehydration and kidney problems
- Infusion reaction; rare serious lung or skin effects

Irinotecan

- Early cramps, sweating and diarrhoea
- Delayed diarrhoea and dehydration
- Low blood counts / serious infection
- Nausea, mouth soreness and fatigue
- Hair thinning or loss
- Rare serious lung or allergic reactions

Folinic acid

- Enhances fluorouracil activity and toxicity
- Rare allergic reaction
- Diarrhoea / mouth soreness in combination

Fluorouracil

- Diarrhoea, dehydration and mouth ulcers
- Low blood counts / infection
- Hand-foot skin soreness
- Cardiotoxicity including chest pain / ischaemia
- Severe early toxicity including DPD-related toxicity
- Pump / central-line complications

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

Panitumumab + FOLFIRI

No routine panitumumab premedication unless previous hypersensitivity/local policy indicates. Written delayed-diarrhoea plan and urgent fever/dehydration advice; do not confuse with acute cholinergic management. Skin care and electrolytes; monthly Mg/Ca/electrolytes for two months after last panitumumab.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00448/

Sources and review status are available online. Follow your own prescription and hospital instructions.